

Achilles Tendon Injury

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Provenance note: This PDF is a local English-text snapshot of the official page. Navigation was removed.
The medical text below was not translated, paraphrased, or supplemented.

The information here will help you understand your injury, guide you through your rehabilitation and explain what you can do to maximise your recovery.

Advice for a recent ankle injury

In the first 72 hours, it's important to:

rest your ankle

elevate it above hip level

apply a cold pack regularly

use crutches and a boot for walking to protect your joint

It's helpful to remember PRICER:

Protect

Rest

Ice

Compression

Elevate

Rehabilitation

Medication

The Emergency Department (ED) or a Minor Injuries Unit (MIU) may have prescribed you with some pain relief. Take these as instructed to help manage the pain.

If you do not feel that this medication is helping, talk to us during your appointment in Fracture Clinic, or your GP, about an alternative option.

Cold packs

Ice is a great natural anaesthetic that helps relieve pain and controls swelling. Apply ice packs or a bag of frozen peas wrapped in a wet thin towel to your injured area for up to 15 minutes every few hours.

Do not place ice in direct contact with the skin.

Do not leave the ice pack on for more than 20 minutes at a time in one hour.

Rest and elevation

It's important to rest your injured area as much as possible in the first 24 to 72 hours.

It's normal to experience swelling after your injury which is often worse at the end of the day.

Elevation reduces swelling, which in turn relieves pain and speeds up your healing. Keep your injured area elevated as much as possible during the first 24 to 72 hours.

If you have been given a sling, use it to keep your injured area elevated. You may find it more comfortable to use pillows to elevate it during the evening and at night.

If you have a lower limb injury, try to keep your feet elevated above the level of your hip.

Smoking

Medical evidence suggests that smoking prolongs fracture healing time. In extreme cases it can stop healing altogether. Stopping smoking during the healing phase of a fracture will help ensure optimal recovery from this injury.

For advice on stopping smoking and local support available, please discuss this with your GP or go to the NHS Better Health website.

Helping your recovery

Being healthy can help you recover from your injury faster. Eating well, staying hydrated and keeping as active as possible will all help. We've put together a set of videos on all the things you can do to help you recover as quickly as possible.

If you need an operation on your ankle

Sometimes an ankle injury may not heal by itself and you'll need an operation to repair the break. Having an operation is always your choice and your surgeon will discuss the benefits and risks of surgery with you so you can make an informed decision. You can find out more about having an operation on your ankle on our surgery for fractures and soft tissue injuries page, and in this video:

Care plan

Below we have a care plan, including information that will help you understand your injury, guide you through your rehabilitation and explain what you can do to maximise your recovery.

Your diagnosis should have been made by an orthopaedic consultant or one of the team working under the supervision of the consultant team. The information in this care plan is to help you understand your diagnosis and is not exhaustive.

Non-urgent advice: Please note:

Your consultant may give you a different, personalised care plan based on your injury and whether you need imaging (x-ray, CT scan, MRI, or an ultrasound) or an operation.

What is your injury?

You have torn or ruptured the tendon at the back of your ankle.

Healing

This injury normally takes between 6 and 9 months to heal. Full rehabilitation can take more than 1 year, although there's no guarantee that you'll get back full function at the end of your rehabilitation.

After this injury, there is a 10-30% chance of re-rupture of the Achilles tendon over your lifetime. This happens because of repeated injury or stress to the tendon during healing. To help reduce the risk of re-rupturing your tendon, it's very important that you follow this care plan and advice from your physiotherapist closely.

Follow up

A specialist physiotherapist will call you about a rehabilitation plan and explain how to adjust your boot. If you haven't heard from us by 4 days after your injury, please call the Fracture Clinic. You may need an ultrasound. If you do, we'll call you and ask you to come in.

If you are under 50 or biologically fitter, you'll also need to be assessed urgently by a consultant or a member of the team in case you need an operation. These operations need to take place within 14 days of your injury.

You can find out more about having an operation on your ankle further down the page.

Please contact the Fracture Clinic on 01872 253091 if:

you're unable to follow this rehabilitation plan

you have any questions

you do not receive your appointment details

you're struggling with your boot

you experience pain or symptoms other than at the site of the original injury or surrounding area

You can also book an open appointment at any time in the six months after your injury. Please call 01736 758892 to make an appointment.

Wearing your boot

You will have been given a VACOped boot by the Emergency Department (ED) or Minor Injuries Unit (MIU). You can see how to fit your VACOped boot in this video.

The boot protects your ankle and will make you more comfortable. You must wear the boot all the time including in bed at night. Always wear a sock.

You'll need to adjust the range of movement your boot allows in the weeks after your injury. Your rehabilitation plan below explains when and how.

Preventing a blood clot or Deep Vein Thrombosis (DVT)

You're not allowed to do any exercises for a number of weeks. This puts you in a higher risk group for developing a DVT which can have very serious effects.

To reduce this risk, you have been prescribed a course of a blood thinner (anti-clotting medication). You'll need to inject this at the same time every day for the first 4 weeks. We'll show you how to do this.

If you have any questions about this, or if you have a reaction to the medication, please speak to your GP or district nurse.

You must dispose of your used injections safely into the yellow sharps bin provided. Once full, you can drop the sharps bin to the Fracture Clinic, your local Minor Injuries Unit or your GP practice where we can dispose safely of them.

Your rehabilitation plan

Your specialist may give you a different rehabilitation plan based on your injury and circumstances.

After your injury:

wear the boot during the day and night

remove your boot carefully for washing and to change your sock once a day. Whilst doing this, you must always keep your toes pointed down as shown in this picture. Do not flex your foot.

do not stretch the calf or do any ankle exercises. This will disrupt tendon healing and your long term recovery.

use your crutches and put some weight through your injured leg when you walk

continue taking anti-clotting medication

After 2 weeks:

adjust the range of your boot to between 20 and 30 degrees plantarflexion.

This video shows you how to adjust the range of motion.

After 4 weeks:

adjust the range of your boot to between 10 and 30 degrees plantarflexion.

remove the boot throughout the day and gently move your ankle up and down within the range allowed by the boot. Do not push through any 'stretch.'

After 5 weeks:

keep the range of your boot between 10 and 30 degrees plantarflexion

change into a flat sole

regularly remove the boot throughout the day. Gently move your ankle up and down within the range allowed by the boot.

After 6 weeks:

adjust the range of your boot between 0 and 30 degrees plantarflexion

regularly remove the boot throughout the day. Gently move your ankle up and down within the range allowed by the boot

rest your feet flat on the floor at an angle of approximately 90 degrees when sitting down

smoothly and slowly raise and lower your heels whilst sitting

After 8 weeks:

remove the boot at night

try to gradually remove your boot during the day – this may take a week or two

try walking short distances around your own home wearing a shoe

wear bilateral heel raise pads, these should be given to you at the time your boot was issued or at a follow-up fracture clinic appointment.

start 'later rehabilitation' in discussion with the physiotherapy team. We will guide you through this at the right time

But do not start any exercises until you have been guided by your physiotherapist.

Boots, support straps, plaster casts and crutches

Using your boot

Your boot protects your ankle and will make walking more comfortable. It won't heal your fracture but it will improve your symptoms.

Taking your boot on and off

Click on the image of your boot below to see a video showing you how to fit your boot.

Footwear for the foot on your uninjured leg

Choose a supportive shoe or trainer with a firm sole for the foot on your uninjured leg. You'll notice that your boot has a thicker sole. By matching this height on the uninjured side, you'll reduce stress on your other joints.

You can buy an adjustable shoe balancer to match the height of your injured side.

Boot advice for people with diabetes

If you're diabetic please contact the Fracture Clinic on 01872 253091 to discuss your boot. It's especially important if you have problems with your skin. We can provide you with a specialist diabetic boot.

Using your crutches

These videos show you how to use your crutches.

Using your ankle support strap

We may have given you a strap to support your ankle, usually if you have badly sprained your ankle or after you've had an operation.

What should I do with my boot, support strap or crutches when I no longer need them?

You can return crutches to the Fracture Clinic or your local physiotherapy department.

We cannot use boots or support straps again. You should not return them to the hospital.

If you've been given a plaster cast

For some ankle injuries the Emergency Department (ED) or Minor Injuries Unit (MIU) may fit you with a plaster cast. Keep this clean and dry. You can find out more about caring for your plaster cast [here](#).

Preventing complications

Caring for your wound and stitches

If you've had an operation, you'll need to keep your stitches dry and clean. You should also look out for any redness or signs of swelling, bleeding or pus as these can be signs of an infection.

You can find information about how to care for your stitches, if/when they need to be removed and signs of an infection on our [surgery for fractures and soft tissue injuries](#) page.

Blood clots and Deep Vein Thrombosis (DVT)

Because your limb is immobilised and you'll be moving less than normal, you're at higher risk of developing a blood clot or Deep Vein Thrombosis (DVT). This can be life threatening if left untreated.

Symptoms of DVT in the leg are:

throbbing or cramping pain in 1 leg (rarely both legs), usually in the calf or thigh

swelling in 1 leg (rarely both legs)

warm skin around the painful area

red or darkened skin around the painful area

swollen veins that are hard or sore when you touch them.

These symptoms can also happen in your arm or tummy if that's where the blood clot is.

More information about DVT

Watch this video about how to prevent DVT.

Learn more about DVT including symptoms, who is at risk and how to treat it.

Find out how to reduce the risk of blood clots when you have an immobilised lower limb (PDF, 108 KB).

Preventative medication for DVT

If you have been told not to put any weight through your leg or to partially weight bear (50%), you should have been assessed in ED or MIU and told if you need to take preventative medication.

If you have not had this conversation please call your GP for an urgent appointment.

Depending on your injury, you may have been prescribed a course of a blood thinner. There are a few different types of medication and they often need injecting into your tummy. You can discuss an alternative with a doctor. Please see the relevant care plan for your injury.

Pressure sores and ulcers

Wearing a boot or plaster cast combined with not moving your limb can lead to pressure sores or ulcers. These are injuries to the skin and underlying tissue caused by prolonged pressure, rubbing or tension.

They often develop gradually, but can sometimes form in a few hours.

Signs and symptoms of pressure sores

These include:

skin discolouration – red patches for people with pale skin, purple or blue patches for people with dark skin

discoloured patches that do not turn white when pressed

a patch of skin that feels warm, spongy or hard

pain or itchiness in the affected area.

If you have a boot, take it off every day to check your skin and make sure it's not rubbing.

If you have a plaster cast, look out for a change in colour (red or white) of your skin above or below the plaster, an increase in pain inside the plaster or pins and needles.

Watch this video to learn how to prevent pressure sores.

Read more about pressure sores on the NHS website.

If you think you have a pressure sore or ulcer call the Fracture Clinic straight away on 01872 253091.

For out of hours advice please call 01736 758909.

Sepsis

Sepsis (also known as blood poisoning) is a serious infection that can be life threatening. There is no one sign for sepsis so it can be hard to spot.

Call 111 immediately if you develop any of the following:

Slurred speech or confusion

Extreme shivering or muscle pain

Passing no urine in a day

Severe breathlessness

It feels like you're going to die

Skin is mottled or discoloured.

Find out more about the symptoms of sepsis on the NHS website.

These videos also explain what sepsis is and how to spot the early signs.

Living with your injury

Work

Decisions to return to work are made on a unique basis and should be discussed with the Fracture Clinic, your GP, and your employer.

You may need time off work and when you return, you may need light or amended duties. When you should return to work will depend on your profession and your injury.

If you see us in person at the Fracture Clinic we can give you a fitness for work statement or a sick note. If you don't come to see us, you can also get one from your GP.

Driving

If you have an injury to a lower limb (foot, ankle, or knee), you can return to driving when you:

have been told you no longer need your brace, boot or crutches, or your plaster cast has been removed

can walk comfortably

can perform an emergency stop pain-free.

Always test your ability to drive with the ignition off and in a safe environment.

If you have a follow-up appointment with us, please wait until your consultant or physiotherapist has given you clearance to drive.

If you're in any doubt, do not drive until you have spoken to our team.

Sport

You should not do any sports at all for at least 6 weeks.

You must avoid impact sports, jumping, running, dancing, and heavy lifting for at least 12 weeks.

Some injuries will mean you have to wait longer before you can return to sport or impact activities. Please check your specific care plan. If you're unsure about whether you should do a sport or activity, please call the Fracture Clinic on 01872 253091.